

Advise the patient with HIV

- Encourage disclosure to supportive partner, family member or friend and refer to counsellor/support group. Advise patient's partner/s and children be tested for HIV.
- Encourage safe sex even if partner has HIV or patient on ART. Advise correct and consistent use of condoms with all partners. Demonstrate and give male/female condoms.
- Explain that HIV is treatable but not curable and needs lifelong adherence to treatment to stay well and to prevent resistance.
- Explain the benefits of starting ART early, regardless of CD4 or stage but especially if CD4 $\leq$ 200, stage 3 or 4, pregnant or breastfeeding.
- If patient chooses not to start ART: identify barriers, link to counselling and review blood results and ART readiness in 1 week. If remains unwilling to start, re-educate about importance of early treatment, refer to wellness programme, and advise to return immediately if s/he becomes unwell.

Treat the patient with HIV**If patient not on ART**

Plan to start or restart **TDF + 3TC + DTG (TLD)**¹, same day if possible (or within 7 days). Follow Steps 1-5 **↪ 114**.

If patient on ART

- If not on a DTG-based regimen, check if eligible to switch same day **↪ 117**.
- Ask about any new medications: especially TB and epilepsy treatment, contraceptives and other common medications like: calcium, iron, zinc, antacids, metformin. If needed, adjust ART or dosing **↪ 118**.

- Give **influenza vaccine** 0.5mL IM yearly. Avoid if CD4 $\leq$ 100. Check that patient is up to date with his/her COVID-19 vaccination.
- Give prophylaxis: TB preventive treatment (TPT), co-trimoxazole preventive therapy (CPT) and fluconazole as needed:

Medication	When to give/avoid	What to give	Side effects	When to stop
TB preventive treatment (TPT)	• Start TPT if not already had TPT <i>and</i> no current TB symptoms. • Avoid if previous RR-TB, neuropathy, liver disease, alcohol misuse. • If TB contact²: start TPT even if already had TPT.	• If already on ART with VL < 50 in last 6 months, give 3HP and pyridoxine ↪ 89. • If starting ART or on ART and viral load $\geq$ 50, give instead isoniazid for 12 months (12H): give 300mg daily. Also give pyridoxine 25mg daily.	• If pain/numbness of feet, peripheral neuropathy likely ↪ 66. • If rash ↪ 67. • If jaundice: refer same day. • If nausea, vomiting, abdominal pain: check ALT and review result within 24 hours ↪ 112.	• If on 3HP, stop after 3 months. • If on 12H, stop after 12 months.
Co-trimoxazole preventive therapy (CPT)	Start if: • CD4 $\leq$ 200 <i>or</i> • WHO stage 2, 3 or 4 disease	• If CrCl > 50, give co-trimoxazole 160/800mg daily. • If CrCl 10-50, give co-trimoxazole 80/400mg daily. • If CrCl < 10, give co-trimoxazole 80/400mg 3 days a week (Mon/Wed/Fri).	• If jaundice: refer same day. • If nausea, vomiting abdominal pain: check ALT and review result within 24 hours ↪ 112. • If nausea/vomiting ↪ 45. • If rash ↪ 67.	Stop after at least 1 year once CD4 > 200, regardless of stage. If started for TB and CD4 > 200, stop after 6 months.
Fluconazole	• If CrAg³ result positive, refer for lumbar puncture (LP). If delay in referral for LP, start fluconazole. • If pregnant/breastfeeding, liver disease, previous CCM⁴, discuss with HIV hotline before starting ↪ 178.	• If delay in LP expected, give fluconazole 1200mg. • If CCM³: once discharged, give fluconazole 200mg daily to complete at least 1 year. • If no symptoms and LP clear: complete fluconazole 1200mg daily for 2 weeks, then 800mg daily for 2 months. Then fluconazole 200mg daily to complete at least 1 year.	• If jaundice: refer same day. • If nausea, vomiting abdominal pain: check ALT and review result within 24 hours ↪ 112. • If nausea/vomiting ↪ 45.	Stop after at least 1 year if CD4 > 200 and VL < 50.

Review the patient with HIV

Visit	Baseline visit	Month 1	Month 3	Month 4	Month 10	6-monthly	Yearly
Note	• Baseline tests - follow up results. Recall patient if needed. • Start ART.	• Review patient. • If well, give 2 months ART. • If unwell, review more often.	• Review patient. • Do viral load and other monitoring blood tests.	• Review results. • Assess for repeat prescription (below).	• Review patient. • Do viral load and other monitoring blood tests.	• Repeat medication script. • Do CD4 if needed.	• Review patient. • Do viral load and other monitoring blood tests.

- Patient eligible to use repeat prescription collection point (RPC)⁵ if: 1) Viral load < 50, 2) Clinically well with no infections, 3) No other uncontrolled chronic conditions, 4) Not pregnant.
- If not eligible or RPC⁵ refused: if patient well enough, give more than 1 month of medication at a time.

¹TDF – tenofovir; ²3TC – lamivudine; ³DTG – dolutegravir; ⁴TLD – fixed combination dose tablet of TDF + 3TC + DTG. ²A TB contact refers to a patient who shared an enclosed space (at work, socially, in a hostel, or in a household setting), for $\geq$ 1 night or for frequent/extended daytime periods, with an adult/adolescent with pulmonary TB ("index patient"), during the 3-month period before the index patient started their TB treatment. ³CrAg - cryptococcal antigen. ⁴CCM - Cryptococcal meningitis. ⁵Repeat prescription collection points (RPC) include 'facility pick-up points' (FAC-PUP), 'external pick-up points' (EX-PUP), clubs. Medications are pre-dispensed by Central Dispensing Unit (CDU) or Central Chronic Medicine Dispensing and Delivery (CCMDD).